

Diagnosis in DIF-Negative Patients

In the few patients with persistent negative direct immunofluorescence (DIF) studies, the diagnosis of bullous pemphigoid (BP) can be accepted if one or more of the following criteria are met:

- **Suggestive clinical picture**, such as tense blisters;
- **Consistent histopathological findings**, specifically subepidermal cleavage;
- **Presence of circulating IgG autoantibodies** binding to the epidermal side of split-skin substrate (SSS) on indirect immunofluorescence (IIF) microscopy;
- **Serum reactivity against BP180 and/or BP230** detected by ELISA or IIF.

Additional Diagnostic Tests

Additional tests may be performed based on clinical context and availability. Refer to Table 1 for further details [25,34–38].

Therapeutic Management (see Tables 2 and 3)

Workup and Pretreatment Screening

A comprehensive pretreatment evaluation is recommended and outlined in Table 2. These recommendations are primarily based on expert opinion and should be adapted according to updated guidelines and local healthcare practices.

Table 2: Suggested Workup and Pretreatment Screening in Newly Diagnosed Bullous Pemphigoid

- Chest X-ray
- Complete blood count (CBC), erythrocyte sedimentation rate (ESR), and C-reactive protein (CRP)
- Creatinine, blood electrolytes, and fasting glucose
- Liver function tests: transaminases, gamma-glutamyl transferase (gamma-GT), alkaline phosphatase, and bilirubin
- Albumin
- Serology for hepatitis B, hepatitis C, and HIV (if immunosuppressive therapy is planned)
- Pregnancy test in women of childbearing age (rare in BP) prior to starting treatment
- Thiopurine methyltransferase (TPMT) testing (optional, if azathioprine is considered)
- Glucose-6-phosphate dehydrogenase (G6PD) testing (if dapsone is considered)
- Exclude serum IgA deficiency (if intravenous immunoglobulin therapy is planned)
- Consider detailed neurological examination and brief cognitive assessment (e.g., Mini-Mental State Examination)
- Evaluate for underlying malignancy based on age, history, and physical findings
- Screen for infections, particularly *Mycobacterium tuberculosis*, when immunosuppression is indicated
- Consider osteodensitometry (DEXA scan) if long-term systemic corticosteroid therapy is anticipated
- Consider ocular examination (intraocular pressure and cataract screening) if long-term corticosteroid use is expected
- Local bacteriological sampling if skin infection is clinically suspected
- Consider echocardiography before initiating systemic corticosteroids, dapsone, or intravenous immunoglobulins

- SARS-CoV-2 infection and vaccination: follow current national and international guidelines and public health recommendations

Objectives of Management

Bullous pemphigoid is a chronic condition that typically lasts several months to years, particularly in patients experiencing multiple relapses [1,2,4,5].

Primary Goals of Therapy:

- Control skin lesions and pruritus, and prevent or reduce the risk of recurrence;
- Improve patient quality of life;
- Minimize treatment-related adverse effects, especially in elderly patients.

Given that BP predominantly affects older adults, comorbidities such as neurological, cardiovascular, metabolic, thromboembolic, neoplastic, respiratory, ocular, and osteoporotic conditions often complicate management [1,2,19,39,40]. Therefore, treatment must be individualized, balancing efficacy with safety.